NCCP 00455 · clinician draft

Pembrolizumab 200 mg · colorectal

Colorectal treatment context This guide covers first-line metastatic MSI-H/dMMR colorectal cancer. The shared NCCP regimen has other cancer indications, but their tests, treatment aims and durations are not this guide.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Pembrolizumab 200 mg every 3 weeks

Active medicines: pembrolizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis or routine infusion premedication is usually required. Use an individual/local symptom-relief prescription if needed.
- Carry the immunotherapy alert card and report possible immune symptoms promptly during and after treatment.
- Effective contraception during treatment and at least 4 months after the last dose.
- For this colorectal context, eligibility is first-line MSI-H/dMMR metastatic disease; PD-L1 is not the colorectal selection test. Do not import a lung adjuvant course cap.
- FBC, renal/liver profile and glucose before each cycle. Thyroid tests every 3–6 weeks: keep thyroid checks when due regardless of the infusion interval. Cortisol as clinically indicated.
- Review any immune toxicity and steroid dose before restarting; no routine dose reduction. Delayed immune effects may occur after treatment stops.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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Sources checked 2026-11-05 · SACTCheck v0.84.35 · Regimen-level drafts; prescriber and oncology-pharmacy verification required